

PROGRESS REPORT & REAUTHORIZATION REQUEST

Applied Behavior Analysis Services

Authorization Period: July 1, 2025 - December 31, 2025

BrightPath ABA Services

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1. Client Information

2. Background & Relevant History

The client is a 4-year-old male who was diagnosed with Autism Spectrum Disorder (ASD), Level 2 (Requiring Substantial Support) at 18 months of age by Dr. Sarah Chen, a board-certified developmental pediatrician. The diagnosis was confirmed through administration of the ADOS-2 (Module 1, comparison score = 8) and developmental history review. A co-occurring diagnosis of Mixed Receptive-Expressive Language Disorder (F80.2) was assigned by Dr. Amanda Torres, SLP-CCC, following speech-language evaluation on 06/15/2023.

The client began receiving ABA services at BrightPath on 01/15/2024. He lives at home with both parents and one younger sibling (age 2). The primary language spoken in the home is Spanish, with English used in community and clinical settings. The client is not currently enrolled in preschool due to behavioral concerns (aggression) but is on the waitlist for an ASD-inclusive program through the local school district.

Relevant medical history includes a history of recurrent otitis media (resolved with PE tubes, 04/2024) and mild constipation managed with dietary modifications. The client takes no psychotropic medications. He receives concurrent speech-language therapy (2x/week) and occupational therapy (1x/week) through a separate provider. ABA treatment goals are coordinated with the SLP to avoid target overlap and ensure complementary communication approaches.

3. Standardized Assessment Results

3.1 VB-MAPP (Verbal Behavior Milestones Assessment and Placement Program)

Date Administered: 10/02/2025

Previous Administration: 04/10/2025

Age Equivalent: 24-30 months

Clinical Interpretation: The client demonstrates a scatter profile consistent with ASD Level 2. Relative strengths are observed in Visual Perceptual Skills (12.0) and Echoic (11.5), which are being leveraged therapeutically to build weaker domains. The most significant deficits are in Intraverbal (2.5), Spontaneous Vocal Behavior (3.5), and Classroom Routines (3.0). Social Behavior (4.5) remains a core treatment target. All domains showed improvement from the prior administration, with the largest gains in Mand (+2.5) and Motor Imitation (+2.0).

3.2 Vineland Adaptive Behavior Scales, Third Edition (Vineland-3)

Date Administered: 10/08/2025 (Comprehensive Interview Form -- Caregiver)

Previous Administration: 04/12/2025

Clinical Interpretation: The Vineland-3 ABC score of 62 (1st percentile) reflects a developmental gap of approximately 26 months. The Socialization domain score of 55 (0.1st percentile, age equivalent 16 months) represents the most significant area of impairment and is consistent with ASD-related social communication deficits. Improvement from the previous ABC of 58 to 62 indicates the client is making gains, though the rate of gain (4 points in 6 months) is slower than would be needed to close the developmental gap without continued intensive intervention.

3.3 CARS-2 (Childhood Autism Rating Scale, Second Edition -- Standard Form)

Date Administered: 10/08/2025

Previous Administration: 04/12/2025

Clinical Interpretation: The CARS-2 raw score decreased from 37.0 (moderate autism range) to 35.5 (mild-to-moderate range). Items contributing most to the current score include Relating to People, Emotional Response, and Adaptation to Change.

4. Skill Acquisition Progress

During the current authorization period, 10 skill acquisition targets were actively programmed. The client mastered 5 targets (50% mastery rate). Detailed progress on each target is presented below.

5. Behavior Reduction Progress

5.1 Aggression

Operational Definition: Any instance of hitting, kicking, biting, scratching, or head-butting directed toward another person's body, with sufficient force to produce an audible sound, leave a visible mark, or cause the other person to vocalize in pain. Includes attempts where contact is made but intercepted by blocking.

Measurement: Frequency (episodes per hour)

Function: Escape from demands; access to tangibles (dual-maintained per FBA dated 01/20/2024)

Intervention Strategies:

- Functional Communication Training (FCT) -- teaching 'break please' and 'my turn'
- Antecedent manipulation -- demand fading with gradual increase
- Noncontingent reinforcement (NCR) -- 2-minute fixed-time schedule
- Differential reinforcement of alternative behavior (DRA)
- Response blocking for safety

Clinical Note: Aggression shows a clear decreasing trend. Spikes correlate with schedule changes and novel demands. The client now independently requests 'break please' in approximately 60% of demand situations where aggression previously occurred. The 2-minute NCR schedule has been thinned from the initial 30-second schedule. Aggression remains above the discharge criterion of <1.0/hr.

5.2 Vocal Stereotypy

Operational Definition: Repetitive vocalizations that are not communicative in context, including scripting from videos/TV, repetitive humming lasting more than 3 seconds, and echolalic utterances not directed at a communication partner. Scored as percentage of 10-second partial intervals during structured instruction.

Measurement: Partial interval recording (10-second intervals)

Function: Automatic/sensory reinforcement (per FBA dated 01/20/2024)

Intervention Strategies:

- Response interruption and redirection (RIRD)
- Differential reinforcement of other behavior (DRO) -- 1-minute momentary
- Increased opportunities for functional vocal behavior

Clinical Note: Vocal stereotypy has decreased but remains above the 20% discharge criterion. The behavior is most prevalent during transition times and low-structure periods. RIRD has been effective in reducing duration of episodes. The team has increased structured vocal responding opportunities (tacting, manding) as competing behavior.

5.3 Elopement

Operational Definition: Leaving or attempting to leave the designated therapeutic area (therapy room, fenced playground, or parent-defined boundary) without adult permission, defined as any movement of both feet beyond the boundary or sustained contact with exit points (door handles, gate latches) lasting more than 2 seconds.

Measurement: Frequency (episodes per hour)

Function: Escape from demands (per FBA dated 01/20/2024)

Intervention Strategies:

- Environmental arrangement -- secured exits with adult supervision
- FCT -- requesting 'go outside' or 'all done' appropriately
- Premack principle -- access to preferred outdoor activity contingent on task completion
- Visual schedule to increase predictability

Clinical Note: Elopement has shown the most significant reduction of all behavior targets. The client now uses 'all done' independently in approximately 70% of situations where elopement previously occurred. Remaining instances are concentrated during transitions between activities and when novel staff are present.

6. Treatment Plan & Goal Status

Goal 1: Communication

Goal Statement: Client will independently use 2-3 word mand frames to request desired items, actions, and cessation of non-preferred activities across 3 settings with 80% accuracy over 5 consecutive sessions.

Baseline Performance: Client used single-word mands for 12 items with gestural prompts in clinic setting only.

Current Performance: Client independently mands using carrier phrase 'I want ____' for 25+ items across 2 settings; 2-word action mands at 55%.

Status: In Progress

Goal 2: Social Skills

Goal Statement: Client will engage in reciprocal play interactions with a peer for 5 minutes, including turn-taking and joint attention, with no more than 1 adult prompt per interaction across 3 consecutive opportunities.

Baseline Performance: Client engaged in solitary play only; no peer-directed social initiations observed.

Current Performance: Client engages in parallel play for 3+ minutes; responds to joint attention at 60%; initiates joint attention at 20%.

Status: In Progress

Goal 3: Behavior Reduction

Goal Statement: Client will reduce aggression to fewer than 1.0 episodes per hour across all settings for 4 consecutive weeks, using functionally equivalent replacement behaviors.

Baseline Performance: Aggression at 8.3 episodes per hour, dual-maintained by escape and tangible access.

Current Performance: Aggression at 2.7 episodes per hour, 67% reduction from baseline.

Status: In Progress

Goal 4: Adaptive Behavior

Goal Statement: Client will independently complete 5 daily living skill routines (hand washing, teeth brushing, putting on shoes, putting on coat, cleaning up toys) with no more than visual schedule support.

Baseline Performance: Client completed 0 of 5 routines independently; required full physical prompting for all.

Current Performance: Client independently completes hand washing and imitation tasks; teeth brushing at 60%; shoes and coat not yet introduced.

Status: Modified

Modification Rationale: Original goal included 7 routines; modified to 5 highest-priority routines after team discussion with caregivers. Shoes and coat deferred to next auth period to prioritize hygiene routines and safety skills.

7. Hours Utilization

Total authorized units: 792 | Total utilized units: 728 | Overall utilization: 91.9%

8. Requested Hours for Next Authorization Period

Authorization Period Requested: 01/01/2026 - 06/30/2026

Clinical Justification: The client's Level 2 ASD diagnosis with co-occurring mixed receptive-expressive language disorder necessitates continued intensive intervention. While significant progress has been made (67% reduction in aggression, 74% reduction in elopement, 5 of 10 skill targets mastered), the client has met 0 of 6 discharge criteria. Ongoing safety concerns (aggression at 2.7/hr), core ASD deficits in joint attention (20%) and social reciprocity, and the need for continued caregiver training support the requested intensity. The Vineland-3 ABC of 62 (1st percentile) confirms substantial adaptive behavior deficits requiring intensive intervention.

9. Caregiver Training Summary

Total Sessions Conducted: 14 (of 24 authorized)

Topics Covered:

- Functional Communication Training implementation at home
- Antecedent strategies for aggression prevention
- 3-step prompting hierarchy for manding
- Visual schedule creation and use for daily routines
- Data collection on behavior frequency (simplified tally method)
- Naturalistic teaching strategies during mealtimes and play
- Response blocking for aggression -- safety procedures
- Generalization strategies across home and community settings
- Toilet training readiness assessment and initial protocol

Caregiver Skill Acquisition: Primary caregiver (mother) demonstrates FCT implementation with 82% fidelity and correct use of 3-step prompting hierarchy with 88% fidelity. Father participates in alternate sessions and demonstrates prompting at 75% fidelity. Both caregivers can independently implement visual schedules for hand washing and mealtimes.

Barriers to Participation: Father's rotating work schedule limits attendance to every other session. Family illness (October) resulted in 4 cancelled sessions. Language barrier -- sessions conducted in English with occasional Spanish interpretation; bilingual materials provided.

10. Medical Necessity Statement

Continued intensive ABA services are medically necessary for this 4-year-old client diagnosed with Autism Spectrum Disorder, Level 2 (F84.0) and Mixed Receptive-Expressive Language Disorder (F80.2).

Functional Impairment: The Vineland-3 Adaptive Behavior Composite score of 62 (1st percentile, age equivalent 22 months) reflects a developmental gap of approximately 26 months. The Socialization domain score of 55 represents the most significant impairment area. VB-MAPP assessment reveals critical deficits in Intraverbal skills (2.5), Spontaneous Vocal Behavior (3.5), and Social Behavior (4.5) -- all domains directly related to ASD core deficits.

Risk of Regression: Without continued services, the client is at significant risk of regression. Aggression, while reduced by 67%, remains at 2.7 episodes per hour. Without ongoing FCT and behavioral programming, regression to baseline levels (8.3/hr) is likely, as replacement behaviors are not yet fully established. Elopement at 0.8 episodes per hour remains a safety concern. Vocal stereotypy at 32% of intervals interferes with learning and would likely increase without intervention.

ASD Core Deficits: Social communication deficits (joint attention initiation at 20%, peer interaction requiring adult facilitation), restricted/repetitive behaviors (vocal stereotypy at 32%), and functional communication limitations (2-word mand frames at 55%) are directly connected to the client's ASD diagnosis.

Intensity Justification: The requested intensity of 24 weekly units of direct service (97153), 4 weekly units of supervision (97155), 2 weekly units of caregiver training (97156), and 1 weekly unit of assessment (97151) is appropriate given the severity of deficits (Vineland ABC 1st percentile), the number of active treatment goals (4 goals, 10 skill targets), ongoing safety concerns, and the client's demonstrated responsiveness to treatment. The client has met 0 of 6 discharge criteria.

11. Discharge Plan

Discharge from intensive ABA services will be recommended when the following criteria are met:

- 1. Aggression below 1.0 episodes per hour for 4 consecutive weeks across all settings (currently 2.7/hr)
- 2. Elopement below 0.3 episodes per hour for 4 consecutive weeks (currently 0.8/hr)
- 3. Vocal stereotypy below 20% of intervals during structured instruction (currently 32%)
- 4. Mastery of 80% of current skill acquisition targets with generalization across 3 settings (currently 50% mastered)
- 5. Caregiver implementation fidelity above 90% for FCT and prompting procedures (currently 82-88% mother, 75% father)
- 6. Vineland-3 ABC score at or above 70 (currently 62)

Criteria Met: 0 of 6

Transition Plan: Phase 1 -- reduce to 20 hours/week direct service with maintained supervision. Phase 2 -- reduce to 12 hours/week focused on social skills and community integration. Phase 3 -- transition to consultation model (5 hours/week) with monthly BCBA check-ins. Final transition to school-based services with IEP consultation.

Current Discharge Readiness: The client has met 0 of 6 discharge criteria. Substantial progress on aggression (67% reduction) and elopement (74% reduction) but not yet at criterion levels. Continued intensive services strongly recommended.

12. Signatures & Attestation

I certify that the information in this report is accurate and reflects the clinical data from the current authorization period. I have personally supervised this client's ABA program and reviewed all data presented herein.

Dr. Maria Gonzalez, BCBA-D
Credential #1-19-42831 | NPI: 1987654321
Date: 11/15/2025

Carlos Mendez, RBT
Credential #RBT-20230198
Date: 11/15/2025

Field	Value
Client ID	CR-2024-0847
Date of Birth	09/14/2021
Age at Time of Report	4 years, 1 month
Primary Diagnosis	Autism Spectrum Disorder, Level 2 -- Requiring Substantial S
Secondary Diagnosis	Mixed Receptive-Expressive Language Disorder (F80.2)
Referral Source	Dr. Sarah Chen, Developmental Pediatrician
Date of ASD Diagnosis	03/22/2023
Authorization Period	07/01/2025 - 12/31/2025
Payor	Anthem Blue Cross California
Member ID	XJK9847201
Supervising BCBA	Dr. Maria Gonzalez, BCBA-D
BCBA Credential #	1-19-42831
BCBA NPI	1987654321
Lead RBT	Carlos Mendez, RBT
RBT Credential #	RBT-20230198
Service Locations	Clinic (primary), Home (secondary)

Domain	Current Score	Previous Score (04/2025)	Change
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Mand	10.5	8.0	+2.5
Tact	7.0	5.5	+1.5
Listener Responding	6.5	5.0	+1.5
Visual Perceptual Skills & MTS	12.0	10.5	+1.5
Independent Play	5.0	4.0	+1.0
Social Behavior & Social Play	4.5	3.5	+1.0
Motor Imitation	9.0	7.0	+2.0
Echoic	11.5	10.0	+1.5
Spontaneous Vocal Behavior	3.5	2.5	+1.0
LRFFC	4.0	3.0	+1.0
Intraverbal	2.5	1.5	+1.0
Classroom Routines & Group Skills	3.0	2.0	+1.0
Linguistic Structure	4.5	3.5	+1.0
Reading	0.0	0.0	0.0
Writing	0.0	0.0	0.0
Math	1.0	0.5	+0.5

Domain	Standard Score	Percentile	Age Equiv.	Previous Score	Change
Communication	58	0.3	18 mo	54	+4
Daily Living Skills	68	2	28 mo	65	+3
Socialization	55	0.1	16 mo	52	+3
Motor Skills	78	7	32 mo	76	+2
Adaptive Behavior Composite	62	1	22 mo	58	+4

Metric	Current	Previous	Change
Raw Score	35.5	37.0	-1.5
Classification	Mild-to-Moderate Autism	Moderate Autism	Improved

Domain	Target	Introduced	Mastered	Current %	Criterion	Gen.	Notes
Manding	Mands for missing items	07/08/25	09/19/25	92%	80%	Yes	Generalized across clinic, home
Manding	Mands for actions using 2-3 word phrases (e.g., 'push me', 'op	09/22/25		55%	80%	No	Emerging with gestural prompts
Tacting	Tacts 50 common objects in natural environment	07/08/25	10/20/25	88%	80%	Yes	Mastered 53 items; generalized
Tacting	Tacts actions in picture videos (e.g., 'running', 'eati	08/12/25		40%	80%	No	Identifies 8 of 20 target actions
Listener Responding	Follows 2-step unrelated actions (e.g., 'clap hands and	07/15/25		35%	80%	No	Follows single-step instructions
Social Skills	Parallel play alongside peers for 3 minutes without disruptiv	09/05/25	11/04/25	85%	80%	No	Mastered in structured clinic s
Social Skills	Initiates joint attention by waving or pointing to items of	09/02/25		20%	80%	No	Responds to joint attention bic
Daily Living	Independent hand washing with visual schedule support	07/22/25	10/15/25	90%	80%	Yes	Completed all steps independ
Daily Living	Tolerates teeth brushing for 30 seconds without crying or tu	08/19/25		60%	80%	No	Tolerates 40 seconds currently
Imitation	Imitates 10 gross motor actions on request	07/08/25	08/28/25	95%	80%	Yes	Mastered all 10 actions; now s

Metric	Value
Baseline (07/01/2025)	8.3 episodes/hr
Current (11/10/2025)	2.7 episodes/hr
Reduction from Baseline	67%
Trend	Decreasing

Metric	Value
Baseline (07/01/2025)	45% of intervals
Current (11/10/2025)	32% of intervals
Reduction from Baseline	29%
Trend	Decreasing

Metric	Value
Baseline (07/01/2025)	3.1 episodes/hr
Current (11/10/2025)	0.8 episodes/hr
Reduction from Baseline	74%
Trend	Decreasing

CPT Code	Description	Authorized Units	Utilized Units	Utilization %	Notes
97153	Adaptive Behavior Treatment by protocol	624	580	92.9%	--
97155	Adaptive Behavior Treatment by Protocol Modification	96	88	91.7%	--
97156	Family Adaptive Behavior Treatment Guidance	68	36	75.0%	Family cancelled 4 sessions due to ill
97151	Behavior Identification Assessment	26	24	100%	--

CPT Code	Description	Weekly Units Requested	Total Units (26 weeks)
97153	Adaptive Behavior Treatment by protocol	24	624
97155	Protocol Modification (BCBA supervision)	4	104
97156	Family Adaptive Behavior Treatment Guidance	2	52
97151	Behavior Identification Assessment	1	26